

WHAT EXPLAINS OCD?

Chemical imbalance in the brain, say experts

WHAT CAUSES OCD?

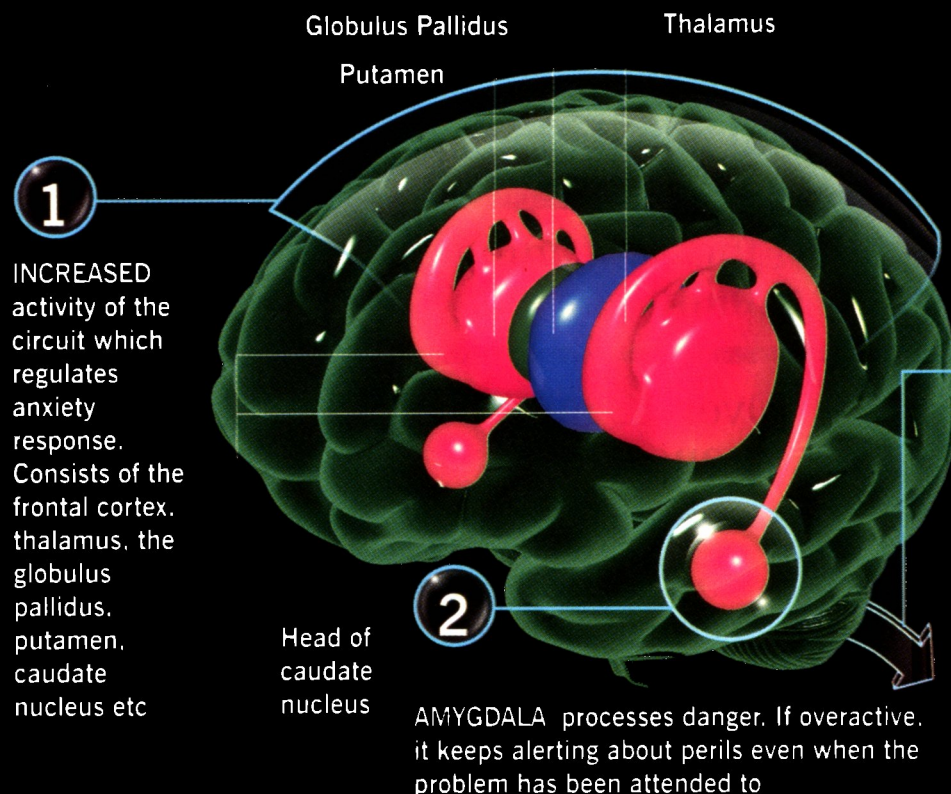
GENES: Can run in the family

STRESS: In about one out of three cases

LIFE CHANGES: Sudden role change—puberty, child birth, new job etc

BRAIN CHANGES: Imbalance of a chemical called serotonin

PERSONALITY: Obsessive, perfectionist, high on morality and responsibility



obsession because the sufferers recognise these as a mental projection and try to ignore those, without much success. It's a compulsion, because the repetitive, seemingly purposeful acts follow rigid rules. "The acts are designed to prevent a future feared event, but are not realistically connected to it, or are excessive," adds Sagar.

And it's a world of suffering. "An OCD patient's anguish is far worse than what we imagine," says Dr Sumant Khanna, who pioneered the OCD clinic at NIMHANS in 1997. "A patient's inner world is torn with fear, guilt, doubt, distasteful images, depression and a sense of defeat." Obsessions and compulsions parade as infinitely personalised variations on a number of morbid themes—from aggression, harm avoidance, contamination, unpleasant or excessive sexual ideation, religious concerns, compulsion to collect, need for symmetry or order, to fear of illness. Contamination and fears of harming oneself or others are the most

New research shows, one in four children with OCD has a first-degree relative with a similar condition

common obsessions, while cleaning and checking are the most common compulsions.

With secrecy and mislabelling, the average lag time between the onset of the disorder and a proper diagnosis is believed to be nine years, according to the US-based Obsessive Compulsive Foundation. It takes an average of eight more years before effective treatment is prescribed. Reddy agrees, "Often patients are in denial and family members force them to come to doctors. Sometimes the inability to function in everyday life, distress and depression force sufferers to seek out doctors."

Lalita Gupta now realises that the whims and quirks of her son Abhishek when he was very young were the first

warnings of an illness that would wreak havoc on their lives. He insisted on wearing only shoes with laces that he could pull very tight. She poo-hooped it all away (Children, after all, do strange things). But when he was in Class V, a classmate was hospitalised with pneumonia. "He heard 'germ', 'infection', and OCD was born." For Abhishek, germs were the initial obsession, but the symptoms soon expanded and grew progressively worse. He feared contamination from soap bars and household cleaners. And then he could not put on his socks. "I would sit on the floor and try to find a pair that would go on," recalls his mother, "but he felt there were stones in them." Finally he couldn't go to school any-